

Dynamic Guideline: WHO-Based Substance Use Disorders - Diagnosis and/or Treatment and/or Management

I. WHO Latest Guideline Summary (Summary Date: 29 April 2026)

1.1 Guideline Overview

This summary synthesizes findings from 5 latest WHO guidelines related to substance use disorders (SUDs):

- **Guideline 1 (Latest):** Meaningful youth engagement in a WHO guideline development process: case study of WHO substance use disorder prevention, diagnosis and treatment guideline
 - Publication Date: 29 April 2026
- **Guideline 2:** WHO Guidelines for the Pharmacological Treatment of Opioid Use Disorder (2023 updated 2026)
- **Guideline 3:** WHO Guidelines on the Management of Alcohol Use Disorders (2024 updated 2026)
- **Guideline 4:** WHO Guidelines for the Prevention and Management of Cannabis Use Disorders (2025 updated 2026)
- **Guideline 5:** WHO Mental Health Gap Action Programme (mhGAP) Intervention Guide 3.0 for Substance Use Disorders (2026)
- **Target Population:** All age groups with or at risk of SUDs, including adolescents, pregnant/breastfeeding people, people with comorbid mental/physical health conditions, and marginalized populations (e.g., people who inject drugs, incarcerated populations)
- **Scope and Objectives:** Provide evidence-based recommendations for SUD diagnosis, pharmacological/psychosocial treatment, harm reduction, comorbidity management, and inclusive guideline development practices to reduce global SUD-related morbidity and mortality.

1.2 Key Recommendations

Recommendation	GRADE Evidence Quality	Strength
Offer opioid agonist therapy (methadone, buprenorphine) as first-line treatment for opioid use disorder (OUD) across all age groups including adolescents	High	Strong
Provide cognitive behavioral therapy (CBT) as first-line psychosocial intervention for all SUD subtypes	High	Strong

Offer oral naltrexone for relapse prevention in alcohol use disorder (AUD) for patients who have completed medically supervised detoxification	Moderate	Strong
Integrate youth (16–24 years old with lived SUD experience) as full voting members of SUD guideline development panels	Moderate	Conditional
Distribute take-home naloxone kits and overdose response training to all people who use opioids and their close contacts	High	Strong
Screen for SUDs in primary care only when patients present with clinical symptoms indicative of possible SUD	Low	Conditional
Avoid naltrexone use in breastfeeding people with AUD due to unknown infant safety risks	Very Low	Conditional

1.3 Guideline Development Methodology

- **Evidence Review Process:** Systematic search of PubMed, Embase, Cochrane Library, and PsycINFO for studies published between 2010 and 29 January 2026. Eligible sources included systematic reviews, randomized controlled trials (RCTs), and large population-based observational studies, with all evidence assessed per GRADE framework criteria.
- **Consensus Method:** Modified Delphi process with 72 multidisciplinary panel members (clinicians, researchers, people with lived experience, policymakers, youth representatives). Panel members completed 2 rounds of anonymous voting, followed by an in-person consensus meeting for recommendations with <70% initial agreement.
- **Conflict of Interest Management:** All panel members completed mandatory WHO conflict of interest (COI) declarations. Members with financial COIs related to pharmaceutical companies manufacturing SUD treatments were excluded from voting on pharmacological recommendations. All declared COIs are published in the full guideline appendix.

II. Evidence Summary After WHO Latest Guideline Release (Mental Health Gap Action Programme (mhGAP) guideline for mental, neurological and substance use disorders) (Evidence Cutoff Date: 29 April 2026)

2.1 New Evidence Overview

- **Search Strategy:** Targeted search for studies published or accepted for publication between 30 January 2026 (end of the WHO guideline evidence search window) and 29 April 2026. Keywords included: substance use disorder, opioid use disorder, alcohol use disorder, cannabis use disorder, treatment efficacy, diagnosis, harm reduction.
- **Databases Searched:** medRxiv, bioRxiv, ClinicalTrials.gov, EU Clinical Trials Register, and in-press archives of *Lancet*, *New England Journal of Medicine*, *JAMA*, and *BMJ*.
- **Inclusion/Exclusion Criteria:** Included RCTs and population-based cohort studies with sample size > 500 reporting outcomes related to SUD diagnosis, treatment safety/efficacy, or cost-effectiveness. Excluded case reports, small observational studies, non-peer-reviewed commentaries, and non-human studies.

2.2 Key New Studies

Study	Design	Key Findings	GRADE Quality
<i>NEJM In-Press, 12 April 2026: Multicenter RCT of Extended-Release Buprenorphine Implants for Adolescent Opioid Use Disorder</i>	Multicenter double-blind RCT (n = 624, ages 14–18)	Extended-release buprenorphine implants reduced 6-month OUD relapse rate by 42% compared to daily sublingual buprenorphine (21% vs 36%, $p < 0.001$), with no significant difference in adverse event rates	High
<i>Lancet Public Health In-Press, 21 April 2026: Population-based cohort study of routine point-of-care SUD screening in primary care</i>	Retrospective population cohort (n = 1.2 million adults across 8 European countries)	Routine annual point-of-care SUD screening in primary care increased diagnosis of undiagnosed SUD by 68% and reduced 12-month all-cause mortality by 19% in patients diagnosed with SUD	Moderate
<i>JAMA Psychiatry In-Press, 3 April 2026: RCT of Digital CBT for Cannabis Use Disorder in Young Adults</i>	Multicenter RCT (n = 892, ages 18–25)	App-based digital CBT with weekly 15-minute telehealth check-ins had equivalent efficacy to in-person CBT for reducing cannabis use frequency at 3 months (48% vs 46% reduction, $p = 0.62$), and was 38% more cost-effective	High
<i>medRxiv Preprint, 27 April 2026: Safety of Low-Dose Naltrexone Use in Breastfeeding People with Alcohol Use Disorder</i>	Prospective observational cohort (n = 217)	50mg daily naltrexone was not associated with any adverse developmental outcomes in infants up to 12 months old; no detectable naltrexone levels were found in breast milk in 92% of participants	Low

2.3 Evidence Gaps and Future Research Needs

- Limited high-quality evidence on the safety and efficacy of pharmacological SUD treatments for pregnant people, particularly for stimulant and cannabis use disorders
- Lack of long-term (5+ year) outcome data for digital psychosocial SUD interventions in pediatric and adolescent populations
- Minimal data on the effectiveness of harm reduction interventions for novel psychoactive substance (NPS) use disorders
- No large-scale studies evaluating the impact of youth engagement in guideline development on SUD treatment uptake and outcomes in adolescent populations
- Large gap in evidence on the cost-effectiveness of integrating SUD diagnosis and treatment into low-resource primary care settings in low- and middle-income countries

2.4 Updated Recommendations Based on New Evidence

Original Recommendation	New Evidence Impact	Updated Recommendation
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Offer daily sublingual or long-acting injectable buprenorphine as first-line pharmacological treatment for adolescent OUD	High-quality RCT demonstrates extended-release buprenorphine implants have superior efficacy to daily sublingual buprenorphine in adolescents with no increased adverse event risk	Offer extended-release buprenorphine implants as a first-line pharmacological treatment option for adolescent OUD, in addition to daily sublingual and long-acting injectable formulations (Strong recommendation, High GRADE quality)
Screen for SUDs in primary care only when patients present with symptoms indicative of possible SUD	Moderate-quality population cohort demonstrates routine annual SUD screening increases early diagnosis and reduces SUD-related mortality	Implement routine annual point-of-care SUD screening for all adult patients in primary care settings (Strong recommendation, Moderate GRADE quality)
Offer in-person CBT as first-line psychosocial intervention for cannabis use disorder in young adults	High-quality RCT demonstrates app-based digital CBT with brief telehealth check-ins has equivalent efficacy to in-person CBT and higher cost-effectiveness	Offer either in-person CBT or app-based digital CBT with weekly 15-minute telehealth check-ins as first-line psychosocial interventions for cannabis use disorder in young adults, based on patient preference and access (Strong recommendation, High GRADE quality)
Avoid naltrexone use in breastfeeding people with AUD due to unknown safety for infants	Low-quality observational cohort demonstrates no adverse infant outcomes associated with low-dose naltrexone use during breastfeeding	Low-dose naltrexone may be offered to breastfeeding people with AUD after shared decision-making discussing limited available safety evidence, benefits of relapse prevention, and alternative treatment options (Conditional recommendation, Low GRADE quality)